CHAPTER 4

Nerves

What was biological psychiatry for doctors was nerves for patients.

Early in the nineteenth century, families brought patients to the asylum not for madness but for imputed nervous illness. Early in the twentieth century, it was nervous illness that provided the platform for private, office-based psychiatry in the treatment of the neuroses. Nerves are, then, central to the history of psychiatry, a great irony considering that diseases of the nerves, properly understood, lie in the province of neurology rather than psychiatry.

Here the social history of psychiatry enters in. Patients found the notion of suffering from a physical disorder of the nerves far more reassuring than learning that their problem was insanity. And until the victory of psychoanalysis in the 1940s, physicians were willing to go along with the fiction that both major and minor psychiatric illnesses were “nervous” in nature. Why psychiatrists were willing to concede to patients this fig leaf is an important issue, permitting us to understand some of the basic forces driving the history of psychiatry. For psychiatrists, the fig leaf of nerves offered a chance to escape the asylum for lucrative private practice with middle-class patients. For patients, this camouflage presented an opportunity to escape the opprobrium of madness and the implications of hereditary illness and degeneration. Unlike psychiatric illness, nervous illness was thought for the most part to be noninheritable and thus non-stigmatizing. In nervous illness, the symbiosis between doctor and patient was perfect: Psychiatrists (meaning the kinds of physicians who would be called psychiatrists after World War II) underwent a big leap up in their social stature and income as they became urban nerve-specialists, electrotherapists, neuropsychiatrists, and the like. And patients experienced the vast relief of knowing that their problems were really “organic” rather than constituting evidence of hereditarian family degeneration or being “all in their heads.”

Nerves Better than Madness

"True!—nervous—very, very dreadfully nervous I had been and am," said the protagonist of Edgar Allen Poe's short story "The Tell-Tale Heart" in 1842, "but why *will* you say that I am mad?"

Why might he be thought mad? For one thing, he could hear from a distance the beating of the heart of the old man he was about to kill: "It grew quicker and quicker, and louder and louder every instant. The old man's terror *must* have been extreme! It grew louder, I say, louder every moment! —do you mark me well? I have told you that I am nervous: so I am."[1]

The pleading of this obviously distracted individual to be considered "nervous" and not insane echoed the pleading of a century, the nineteenth century as the nervous century. It is initially in the horror of the asylum and of the alienist that we can understand the insistence of the population on nervous disease.

From the beginning, the notion of being confined in an asylum evoked fear and loathing among the public. How could it have been otherwise with stories such as that of poor William Norris at Bethlem chained up for 10 years.[2] "The public asylum in Vienna," said Bruno Goergen in 1820, owner of a select private asylum, "just as perhaps all public institutions of this kind, is detested so widely in public opinion that it is scarcely necessary for me to enlarge upon the reasons for this detestation." [3] Public laments resounded down the decades: the asylum as a place of unlawful confinement, of abuse, and of scenes of horror. The alienists' vehement protestations that virtually no sane person was ever confined, or that psychiatrists were honorable physicians and not beasts, were unable to prevail against impressions created by such novels as Charles Reade's *Hard Cash*, published around 1863.

In *Hard Cash*, the wrongly confined Alfred attempts to bribe the attendant with a hundred pounds to let him out:

> A coarse laugh greeted the proposal. "You might as well have made it a thousand when you was about it."
>
> "So I will," said Alfred, "and thank you on my knees besides." Alfred protested that he had the money.
>
> "Well, you are green," replied the attendant. "Do you think them as sent you here will let you spend your money? No, your money is theirs now."[4]

As the air filled with talk of degeneration after the 1860s, this fear of the asylum became a desperate panic to avoid at all costs the confinement of a relative in an insane asylum. The parents of a deeply melancholic young man in Zurich were, in the 1890s, anything but sympathetic when it was suggested to them that their son needed institutional care: “It’s inconsiderate of him not just to pull himself together ... because his three sisters are just coming into marriageable age and their suitors will surely be frightened off. Nobody wants to marry into a family with mental illness.” Another set of parents decided to act only after their son had lain motionless in bed for three years, never speaking a word during this time. Still other parents summoned the resolution only after noting, as they told the clinic, “Our son imagines that he is a horse.”[5]

The doctrine of degeneration meant that it was not merely the asylum that threatened the family and its honor but the psychiatrist and his menacing diagnoses. After the 1860s, the profession’s conventional terms for psychiatric illness began to ring with awfulness. The notion that major psychiatric disorders came from poisoned heredity instilled in the public a kind of permanent dread. Never tell patients they have hypochondria, counseled the owners of a private nervous clinic in Austria: They will look it up in the dictionary and discover that it means “insanity.”[6] Sir Andrew Clark, a distinguished internist at the London Hospital, was said in giving an opinion to have dropped the word “melancholia.” “The outcome of that visit was disastrous, entailling serious trouble all round, in which even Sir Andrew himself shared, for he was pestered for weeks with letters to know whether in using the term ‘melancholia’ he had the idea of insanity in his mind.”[7]

In Germany, turn-of-the-century psychiatrists were appalled at public benightedness in shunning the “odium psychiatricum.”[8] People were willing to credit fantastical stories in which madmen switched identity with mayors. As a psychiatric editor recounted one such story: The Herr Bürgermeister was supposed to transport a madman to the asylum. “Underway he is said to have gone to sleep after a few glasses of beer, whereupon the patient exchanged papers with him. At the asylum the patient is reported to have legitimated himself as the mayor, handing over his horrified companion to the care of the institution. Only after a few days of greatest desperation is the misunderstanding said to have been clarified.” Such reports actually circulated in the German popular press, fortifying the impression that psychiatry meant danger.[9]

It comes therefore as no surprise that turn-of-the-century Germany already had its own antipsychiatry groups. “What do we want?” asked the budding movement for the Reform of Psychiatric Law and Psychiatric Treatment in 1909. “We want to damn secretiveness, deception, and hypocrisy as unworthy of humankind.” Its journal was filled with tales on the order of, “How Beuthen City Councilman Lubecki was Tortured in the Madhouse.”[10]

With the press overflowing with stories of madmen switching roles with mayors and city councilpersons tortured in asylums, neither in Germany nor elsewhere did psychiatry have a terribly bright future. Unlike the rest of medicine, said Freiburg psychiatrist Alfred Hoche in his memoirs, the patient sees the doctor not as a friend but an enemy. Young doctors going into psychiatry had to understand, said Hoche, “that their relationship to the patient becomes a quite different one. Normally, patients see the physician as helper, because they hope he will remove their symptoms and effect a cure. But here they reject the idea that they are ill and need a physician’s help. Indeed the doctor in their opinion is often the enemy.” Then Hoche added an interesting observation: “This situation is not so apparent in the psychiatric clinics that treat nervous patients.”[11]

It was not difficult to figure out that if psychiatry was to have a future outside the asylum, it must be under the rubric of nerves, and not under the proper descriptive term psychiatry. The latter had come to signify inexorable hereditary madness, while nervous illness implied merely an organic affliction of the nerves not necessarily hereditary in nature and not dangerous to the marriage prospects of one’s daughters. This insight dawned on practitioners almost from the founding of the discipline itself, and the vast majority of psychiatric care in past times was delivered under the label nervous illness rather than psychiatry.

Although the category of nervous illness as a variety of psychoneurosis dated back to Cheyne in the 1730s, only in the first decades of the nineteenth century did nerves start to become a euphemism for psychosis. From the 1830s on, psychiatric illness—or “insanity”—became increasingly translated in the public mind as nervous illness. And psychiatrists in their communications to the public tended to use nerves, nervous, or some variation on this theme when they meant organic, brain, biological, constitutional, or some variation on that theme. This was a massive duplicity, a century-long deception of the public to the effect that illness meant a disorder of the nerves when in fact the brain was meant. While the patients

believed their “nervous” problems stemmed from overwork (among the middle classes) and humoral imbalances (among the lower), doctors believed nervous problems to be constitutional in nature and possessing a heavy genetic component. Although both doctors and patients used the same terms, the malentendu could not have been more complete.

Why did this kind of deception—the kind of deception that occurs today when doctors tell patients they are suffering from “stress”[12]—seem necessary? There is always great pressure on physicians to tell patients what they want to hear, a pressure that affects psychiatrists in particular where an upbeat mood is supposed to be the outcome of therapy. What Bernard Shaw said in 1911 about the doctor-patient relationship in general was not necessarily intended for psychiatry, though it might well have been:

> The doctor who has to live by pleasing his patients in competition with everybody who has walked the hospitals, scraped through the examinations, and bought a brass plate, soon finds himself prescribing water to teetotallers and brandy or champagne jelly to drunkards; beefsteaks and stout in one house, and “uric acid free” vegetarian diet over the way; shut windows, big fires, and heavy overcoats to old Colonels, and open air and as much nakedness as is compatible with decency to young faddists, never once daring to say either “I don’t know,” or “I don’t agree.”[13]

The inference is that nineteenth-century psychiatrists felt themselves under much duress to convey to patients notions of the cause and nature of their illnesses that the patients wanted to hear.

“I was termed a nervous patient,” said John Perceval, an English gentleman admitted in 1831 to the Brisslington private asylum. Perceval had been hearing voices and seeing visions. His mother “suffered from extreme nervousness” too, “during which she could scarcely endure, and even forbade a newspaper being unfolded in her room, so greatly did she feel the need of quiet.”[14] Glad to accommodate such wealthy families as the Percevals, in 1835 Evans Riadore, a Harley-Street doctor in London, called nervous affections specially common “amongst the higher classes of society.” These disorders “so embitter their lives, as to render their portion of worldly enjoyment nearly on a level with that of the poor and laborious.”[15]

French writers chose to speak in these years not of melancholia—a psychotic illness apt for the asylum—but of “nervous erethism,” a term

encompassing irritability, emotional instability, and depression. Such erethism, situated particularly among urbanites accustomed to “luxury,” was said to be on the rise.[16]

In Germany, with its large concentration of private clinics, this vogue for nervous illness began showing up early in institutional name changes. Dr. Adolf Albrecht Erlenmeyer’s private asylum, founded in 1847 as a “Private-Institution for the Insane and Idiots,” had 10 years later become a “Private Institution for Brain and Nervous Disease.” In 1858, Dr. Erlenmeyer explained that his clinic was for “fully-developed forms of mental disturbance (curable and incurable) as well as for early forms, sufferers of which are starting to be designated with the name nervous patients *[Nervenkranke]*.”[17] Sometime between 1858 and 1876, the private asylum in Eitorf in the Rhine province changed the wording of its admission policy from “insane only” to “nervous patients.”[18] Were these changes the result of science on the march? As Ewald Hecker, who by 1881 had terminated his association with Karl Kahlbaum and was running his own private clinic for “nervous patients” in Johannisberg-on-Rhine, put it, “Among insiders it is an open secret that this designation has been chosen only euphemistically, in order to make it easier for the relatives of an insane patient *[Geisteskranke]*, to whom the name insane asylum appears horrifying, to bring the individual for admission.”[19] By 1900, virtually every major private psychiatric clinic in Central Europe had abandoned such terms as “mentally disturbed” (*psychisch-gestört*) and “insane” (*Irre*) in favor of self-descriptions involving disorders of “nerves” and “mood” (*Gemüt*). In the private sector, the fiction was complete.

Such was the public’s revulsion from “psychiatry,” that even in the public sector in Central Europe old-style descriptions were dropped for new. In 1906, psychiatry professor Robert Sommer in Giessen was able to persuade the health ministry to accept a name change for his university psychiatry department from “Clinic for Insanity” to “Clinic for Psychiatric and Nervous Disorders.” He wished the change, he said, “out of consideration for the psychiatric and voluntary nervous patients, who did not wish to be considered ‘mentally ill’ *[Geisteskrank]*.”[20] When Ferdinand Kehrer was able to establish a “neuropsychiatric clinic” in Münster in 1924, “it was for the sake of the public that we simply called it a ‘Nervous Clinic’ *[Nervenklinik]*.”[21]

In England, there was no suggestion of naming comparable clinics “psychiatric.” When a short-stay psychiatric hospital and outpatient

department were added to the Edinburgh asylum at Morningside in the 1920s, it was decided to name the place "Jordanburn Nerve Hospital and Psychological Institute." Psychiatric wasn't even in the running; one authority said that even the term "nervous" has acquired an unfortunate "popular meaning or at least suggestion. But that is not fatal, and I should prefer 'nervous' to 'psychopathic,' which is horrible." The chief physician nonetheless put "nerves" in ironical quotation marks, for he knew very well what he was treating.[22]

All this renaming occurred for no underlying scientific reason. Doctors understood Griesinger's "nervous disease" to mean biological brain disease rather than disease of the peripheral nerves. The renaming was done for the sake of the public. The asylums in Germany continued to do the classic heavy lifting of psychiatry: psychosis. "Yet the public wants to call them 'nervous'-clinics, in order to avoid that harshly ringing term 'insane'-asylum or that foreign term psychiatric," as one asylum psychiatrist said after World War I.[23]

The flight from psychiatry also spilled over to internal medicine and general hospitals: Anyplace but the asylum! the families were crying. What is the logic for separating neurology from psychiatry? asked asylum psychiatrist Paul Näcke, who wanted them separate. "Nervous patients in particular find it undesirable to be on a psychiatric ward, even in separate units or in special houses, because it is well-known that many nervous people find the immediate presence of the mentally ill oppressive and start to be filled with dread, thus delaying their own recoveries." Näcke pointed out that having psychiatric units in general hospitals would let someone with an "acute psychosis recover unnoticed, without being branded in the public eye with the mark of shame of the asylum."[24] It had thus become impossible to practice psychiatry in a straightforward manner. Physicians wishing to treat major psychiatric illness among the middle classes, and minor psychiatric illness among any group of the population whatsoever, would have to find settings that fictionalized the nature of the illness.

The Flight of Madness into the Spa

Before the advent of the modern hospital around the time of World War I,

the middle classes sought their health care at watering holes and spas. There were many reasons for this. Since ancient times, water itself had been assigned a therapeutic virtue, and the particle content of the mineral springs did help to get the bowels moving and overcome constipation. Some springs rich in iodine or iron would have relieved the medical conditions caused by shortages of those substances. The placid routine of the spa doubtless allowed many to unwind from the pressures of business and sociability. All of these factors help to explain why the custom of visiting spas for psychiatric conditions had remained intact since the eighteenth century (see pp. 23-24).

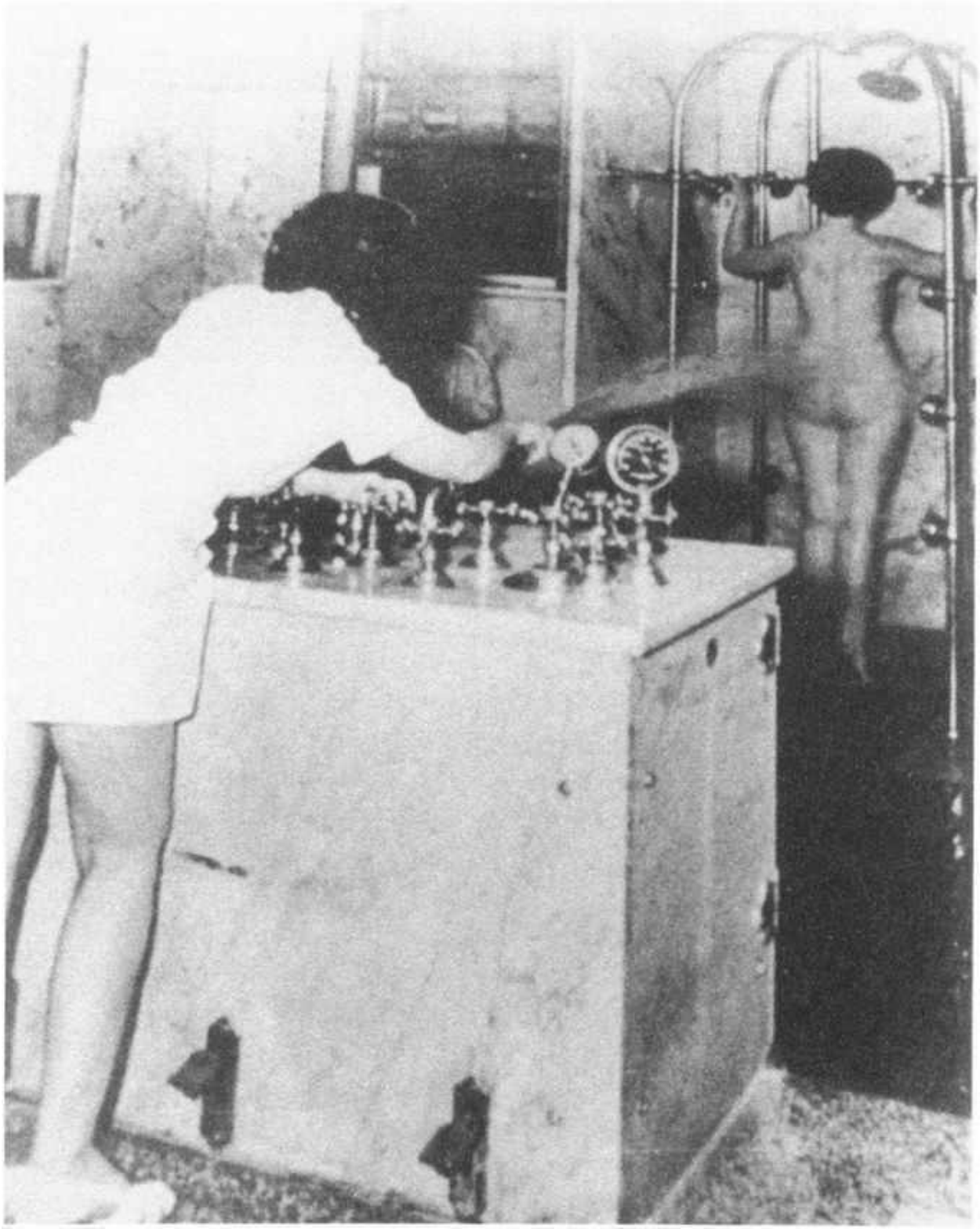
Yet in the nineteenth century, the frequency of spa-visiting vastly accelerated. Moreover, hydrotherapy became expressly associated with psychiatric illness rather than being a panacea for every condition imaginable. Here again, numerous social changes of the 1800s help account for this new popularity: the rising wealth of the middle classes (for spa-visiting previously had been a privilege of the nobility); middle-class interests in social improvement that translated ineluctably into self-improvement and into a heightened fascination with one's own body; better means of transport, particularly the extension after 1860 of the secondary rail network to small towns and villages having mineral springs. Demand soared. Water-therapy clinics sprang up, run by physicians who were able to give a commercial focus to the entire experience of spa-going. All of these circumstances operated in every country in the Atlantic community, making the spa by 1900 the elective middle-class site for the treatment of chronic illness. As the particular chronic illnesses addressed by spa therapy became increasingly psychiatric, the spa became the first place of refuge from the asylum.

The vogue for spa therapy was perhaps briefest in England. Although English invalids had always sought out such watering places as Bath, only in the 1840s did English spas become a medical rage in the form of "hydros," private clinics to which the middle classes could repair for comprehensive treatment. Joseph Weiss, an Austrian veteran of spa therapy, established the first English water-cure clinic around 1841 in Stansteadbury in Hertfordshire, followed then by two Englishmen—James Wilson and James Gully—who opened for business in Malvern the following year.[25] By 1850, at least two dozen of these establishments existed in Britain. There is no doubt that among the thousands who flocked to them were a number of patients with some kind of major psychiatric problem, a trend no doubt accelerated by the

passage in 1843 of an Asylum Bill that made it difficult for private asylums to admit patients on a voluntary basis: These patients would now seek other kinds of care where they had the assurance of being able to check out again voluntarily.[26] Charles Darwin's visits to Malvern for his chronic hypochondria are well-known. For a while, Edward Bulwer-Lytton, first Baron Lytton and well-known parliamentarian and author, also obtained relief at Malvern for symptoms that he considered nervous in nature:

> Bulwer-Lytton, as he tells us in 1846, had been quite ground down by work, yet rest was of no avail. In his attempts at repose, "all my ailments gathered round me—and made themselves far more palpable and felt. I had no resource but to fly from myself—to fly into the other world of books, or thought, or reverie.... As long as I was always at work it seemed that I had no leisure to be ill. Quiet was my hell."
>
> Then Baron Lytton began developing frank psychiatric symptoms as well. "The exhaustion of toil and study had been completed by great anxiety and grief." After the death of a family member, "I seemed scarcely to live myself." By January 1844, he was "thoroughly shattered." The least exercise exhausted him. "The nerves gave way at the most ordinary excitement," a result he believed of "a chronic irritation of that vast surface we call the mucous membrane." The obviously despondent Baron Lytton, like so many of his countrymen, was restored by the healing procedures of the hydro at Malvern.[27]

Before the advent of the physical therapies in the 1930s, hydrotherapy was one of the few means of calming agitated patients. Here the Mississippi State Hospital, Whitfield, c. 1920 (courtesy of Mississippi State Hospital).



Yet the English fashion for local hydrotherapy was relatively brief. In the

1880s, wealthy English nervous patients, spurning domestic spas as impuissant or unchic, began searching out those on the Continent (since the 1820s the English had been wintering on the Continent for chest diseases). The vogue for nerves was new, and unwelcome to the foreign consultants who depended on chest complaints for their livelihood. [28] But with the efforts of the French in the 1880s to downplay tuberculosis (TB) (then known to be contagious) on the Riviera and to highlight nerves, English medical opinion lost its parochialism.

Leaders in this change were two generations of a Harley Street medical family named Weber, quintessential examples of physicians who, though apparently general internists (or the equivalent at the time), devoted much of their practice to psychiatry. The father, Hermann Weber, had been born in Germany and studied in the late 1840s under his uncle the Bonn psychiatrist Friedrich Nasse. After encountering Thomas Carlyle in the early 1850s in Bonn, Weber decided to visit England. There he married an Englishwoman and decided to stay on, able to practice medicine after becoming a licentiate of the Royal College of Physicians of London in 1855. Owing to his great personal charm, he acquired a carriage-trade practice, became physician to five prime ministers, and styled himself as a climatologist, a specialist in spas. In his widely read guide to spa therapy, published in 1880, Sir Hermann advised his medical readers to recommend the Italian Riviera for nervous disorders.[29] By 1898, when Weber senior and his medical son Frederick put out another edition of their guide, they had become much more favorable to the high country overlooking the Riviera beaches: “For neurotic patients and those suffering from neuralgic conditions, neighbouring more elevated regions are mostly preferable, such as Grasse and Cimiez.”[30]

The records of Frederick Parkes Weber’s extensive Harley Street practice have survived, and it is evident from them that many of the patients he was sending to these Continental spas had some kind of psychiatric problem. A not untypical example was Miss X, 25, of Durham, who had come down to London in October 1908 to consult Parkes Weber (as he was called, pronounced VAY-ber) about occasional pain around the site of an appendectomy she had had eight years ago. Parkes Weber detected a more immediate problem, however: This “rather slight, rather pale” young woman would vomit continuously after meals for about an hour. She was also “subject to headaches (and bursting sensation in head) this year.” Parkes Weber diagnosed “nausea of incertain cause and mental depression,”

advising her to visit Friedrich Dengler's and Anton Frey's nervous sanatorium in Baden-Baden, or the Val-Mont, another nervous "san" in Territet, Switzerland, "with a winter season afterwards at St. Moritz."[31] It was thus under the influence of such consultants as the Webers that English patients with psychiatric conditions beneath the level of florid psychosis were diverted to watering places on the Continent.

England's own spa business underwent a fatal decline during World War I, never again to recover, at least as regards the carriage trade. In 1922, the special spa trains to Scotland that had run during the War were discontinued, now that "it is cheaper to visit a French or Swiss spa than to make this journey to Scotland," as a medical journal noted.[32] "Let there be no mistake about it," said one London society doctor, Continental spas were superior. "At the best of these establishments [on the Continent] treatment is carried to the nth degree."[33]

In France, spa therapy and water-cure clinics maintained their popularity far longer, partly because the country is blessed with a sunnier climate and a greater number of mineral springs, partly because the faith of the French in the curative power of waters that bubble from the deep was so fervent as to have survived to this day. "Thermal fever" began in the 1820s, the annual number of health-seekers accelerating from 31,000 in 1822 to 100,000 in the late 1830s, to 200,000 in the late 1860s. By the end of the century, French spas were receiving 300,000 to 400,000 visitors a year, and such watering-places as Aix-en-Savoie and Vichy achieved worldwide reputations.[34] These "rustic republics of water-drinkers"—the phrase comes to us from an account of a young woman suffering hysteric fits that later, at a spa, turn into multiple-personality disorder[35]—breathed the spirit of anxious hypochondria and neurosis.

French spas swam in nervous illness. Octave Mirbeau, in his 1901 novel "The Twenty-One Days of a Neurasthenic," allows spa "Dr. Triceps" to cry out, "Névrose! névrose! névrose! ... Tout est névrose!"[36] At Royat in the Puy-de-Dôme department, the waters were said to help mainly "neuropathic manifestations" such as migraine, myalgia, and "certain psychopathic problems." Above all, according to Fernand Levillain, a hydro- and electro-specialist in Nice who consulted in Royat, the spa "offers the most help in neurasthenia." "Today [1894] this disorder has become extremely frequent and it is not rare to encounter isolated symptoms of neurasthenia in the

majority of our rheumatic and gouty patients."[37] Thus Levillain, on the face of it a physical therapist with numerous orthopedic patients, was performing in Royat the duties of a psychiatrist.

If the fetish of English psychiatry was nonrestraint, that of the French was the notion that spas could be finely graded on the basis of indication, or presenting complaint. In the finest Cartesian manner, French spas in their hundreds were classified on the basis of what malady they were apt for. The therapeutic indications embraced all of medicine, from engorgement of the uterus, for which the spas at Salins in the Jura, Salies de-Béarn, and Lamotte were said to be good, to engorgement of the liver, for which Bourbonne and Balaruc were recommended. Among this landscape of pseudo-uses, the indications that ranged above all others were psychiatric. If the patient was cachectic as a result of depression, the waters of Royat, Saint-Nectaire, Sainte-Marguerite, and Châteauneuf would suit. If nervous patients had stomach pain (enteralgia), they should try Néris, Bagnères-de-Bigorre, and Plombières. If "nervosisme" was the problem, Luxeuil and Luchon were indicated. If your nervous patients, doctor, were simultaneously hysterical, send them to Saint-Sauveur, Evian, and Ussat. On and on went these lists, through neuralgia, "softening of the brain," and paralysis: Paralysis from tabes was assigned to Lamalou (where to this day a statue of Charcot sits in the town square); paralysis from hysteria, Olette.[38] With these ultrarefined "indications," the spa-physicians of France put on a spectacular performance of making distinctions without a difference, for most of the healing waters, consumed today in bottles on the dinner table, were placebos. Often dictated by the physician who had the medical concession in the spa at that moment, the supposed indications were nothing more than a triumph of public relations.[39]

Spas in France and French-speaking Switzerland benefited greatly from the shift away from tuberculosis and toward nervous illness that began in the 1880s. The French Riviera suddenly discovered that its climate was indeed much more suitable for nerves than for TB.[40] Montreux experienced a huge increase in custom, from 22,000 visitors in 1896 to 62,000 in 1908, as it shifted indications from tuberculosis to nervous patients, "be they confirmed neurasthenics or patients suffering simply from exhaustion and overexertion."[41] The whole high Alps of France and French-speaking Switzerland opened up to nervous diseases as the tubercular were shunted into such highly circumscribed resorts as Leysin. By 1900, francophone

Europe had become much more accommodating to the nervous than it had been a century earlier.

As in England, the medical center of a typical French watering hole became the private hydros, or water-cure clinics, that sprang up everywhere. Here the psychiatric essence of spa therapy saw its most perfect expression. Although hydrotherapy clinics were found throughout provincial French spas, the greatest concentration—some 40 in number by the turn of the century—was in Paris. It was in these hydros, where tap water and not mineral water was used, that the Parisian middle classes with diagnoses such as neurasthenia and hysteria congregated, exactly the same segment of the population that 70 years later would be seeking psychoanalysis. Typical was the *clinique hydrothérapeutique* of Alfred Béni-Barde, a hydrotherapist of four decades' experience who by the turn of the century had shifted from central Paris to the chic suburb of Auteuil. He described two patients who consulted around 1908: "[They were] twin sisters, one of whom offered the principal traits of hysteria, the other those of neurasthenia. Born in an aristocratic family tainted by numerous stigmata of nervosity and arthritis, they received an eccentric upbringing that exercised a disastrous influence on both of them." The one was sweet, sad, and contemplative, inclined to seek seclusion. She seemed to experience mental gaps and inexplicable lapses. The other was lively, alert, and loved the dirty stories told by her brothers. She was, however, subject to hysterical fits.

Béni-Barde adopted the appropriate hydrotherapeutic procedure for each. "For the neurasthenic we had recourse to a sedative shower, slowly moving over to more bracing temperatures. To treat the hysteric it was necessary to employ the Scottish douche [alternately hot and cold], the cold water obliging her from time to time to dive into a swimming pool filled with chilly water." Since these treatments, said Béni-Barde, both have been healthy, "though retaining a certain impressionability."[42]

Béni-Barde's theory of psychohydraulics was very straightforward: "The hybrid neuropathy that has seized [hysterics] does not require calming. These female patients must be tamed. That is why cold water succeeds."[43] Thus did the hydrotherapists articulate their theories of psychodynamics.

In the years before World War II, the French spas shared to some extent in the fate of the English. Both shed their upper-crust clientele to the great German *Weltbäder*, or internationally celebrated watering places.[44] In Central Europe, the tradition of hydrotherapy for psychiatric reasons ran broad and

deep. Although the German spas reached back to the Middle Ages, it was the nineteenth century that saw them grow dramatically as the urban middle classes took up spa-visiting. And what the new spa guests brought was not fever, not acute infectious illness as the aristocracy had before, but neurosis. In the Bohemian watering place of Karlsbad (Karlovy Vary), for example, the percentage of nobles among the visitors declined from 32 percent in 1793, to 11 percent in 1814, to 1 percent in 1911. The representation of businesspeople, by contrast, rose from 12 percent in 1814 to 59 percent in 1911.[45] The period of most rapid growth in the spas occurred after 1870 with the rising incomes that industrialization brought in tow and the expansion of the railnet. Between 1871 and 1911, visitors to Kissingen more than quadrupled, from 8,000 a year to 34,000; those to Wiesbaden, one of the main "nerve" baths, rose from 60,000 in 1871 to 127,000 in 1900; the number of "cure-guests" at Baden-Baden, another major nerve bath, was up from 50,000 in 1871 to 79,000 in 1911.[46]

The link between spa life and psychiatry established itself early. So puissant were the waters in Baden bei Zurich said to be that in 1818 one physician discouraged their use in patients who had "very weakened nervous systems, if drinking the waters increases the number of nervous episodes and causes prolonged nervous headaches."[47] Such defeatist notes, however, were soon drowned out by cries of jubilation. In a tour of the spa horizon in 1837, one doctor preached the virtue of Gleissen for "nervous weakness," another that of Teplitz for disorders of the "nervous system," specially for hysterical paralyses in young women. Meinberg's waters were said to be splendid for "hysterical incidents," again in young women, and Karlsbad's marvelous for hysteria in general.[48] What was any of this if not office-practice psychiatry?

Major psychiatric illnesses came to the spas as well though they were unwelcome. Until the appearance of the "open" private nervous clinics in the 1860s (meaning patients could be admitted without a certificate), alienists regularly sent psychotic patients to spas. Among the 7,063 patients seen personally by the local spa-doctor at Oeynhausen between 1858 and 1879, 2,111, or 30 percent, had psychiatric and neurological problems of some kind (neurosyphilis being the largest single category). Among those 2,111 there were 118 psychotics. Their number was not great relative to the total of nerve patients, but one imagines the significance for little Oeynhausen over those two decades of 118 individuals having hallucinations and delusions or who were severely thought disordered. The spa-doctor, however, reported good

results even with them, particularly in cases caused by "previous masturbation."[49]

The actual spa therapy of the psychoses and neuroses would have been conducted not in the public bathhouses but in the hydros (Wasser-*heilanstalten*), private residential clinics devoted ostensibly to water treatment but in reality offering a number of other procedures as well—electrotherapy, massage, and the like. These clinics multiplied the various therapies to differentiate themselves from competitors in the fierce struggle for wealthy patients. Among the first Central European hydros was that established by Johann Christian Reil, one of the founders of modern psychiatry, at Halle in 1809. (Among Reil's early patients was the storyteller Wilhelm Grimm.[50]) Yet the craze for water-therapy clinics in Central Europe began only after 1833 as Vincenz Priessnitz, the nonmedical son of a peasant, opened a few lodgings in Gräfenberg, Austrian Silesia, for patients seeking water therapy. Thus began a movement tantamount to a secular religion on behalf of curing oneself through the application of damp packings, partial baths (arm, leg), and full baths in the cold waters of Gräfenberg.[51] The original operators of the hydros in England had been acolytes of Priessnitz, as were an entire generation of lay hydrotherapists and naturopaths who set up shop all over Central Europe.

Originally, the main candidates for these cold baths seem to have been patients with fevers, and the cold water did procure some subjective relief. But as time went on the febrile patients gave way to nervous ones, and the cold waters—more than a weakened nervous constitution could stand it was said—were warmed up. After Priessnitz died in 1851, the fashionable water-therapy centers shifted from the cold springs of the east, run by laity, to the warm springs of Baden-Baden, Wiesbaden, and other spas in the west, run by physicians.[52]

From the outset, the fashionable hydros of Vienna, Lake Constance, the Black Forest, and the Rhine Valley had a heavy psychiatric component. Psychiatry lurked in the background of even the most organic-appearing of them. Wilhelm Winternitz, was professor of hydrotherapy at the Vienna University and founded in 1865 a hydro in Kaltenleutgeben outside of Vienna. He prided himself on having put water treatments on what he considered a scientific basis. Yet Winternitz had previously worked as an asylum physician.[53] The Winternitz hydro included nerves in its advertised indications. It did not advertise for psychotics, although it got them too.

Thus early in November, 1894, Barbara T, age 41, checked into the Winternitz hydro with a feeling of "jerking in the limbs." There she received mild electrotherapy, becoming in the process "increasingly more agitated." She "holds long monologues, attempting to speak High German. At night she prays for hours. Turns her back when spoken to. In a loud voice she incessantly discusses events of her daily life, addressing an inkwell or a stone. She suddenly tries to take off her clothes. Demonstrates erotic desires while being investigated," noted the physicians of the Vienna Asylum after Barbara T had been removed from the Winternitz hydro with a diagnosis of "mania."[54]

The world of the hydros was filled with Barbara T's, to the dismay of asylum psychiatrists who saw such hydro referrals as malpractice. Said one exasperated psychiatrist at a meeting in 1874, "The water-cure clinics and mineral spas do not represent appropriate treatment for patients with early signs of major mood disorder." Yet he noted that, "in the course of the previous year, at various places in Germany and Switzerland several water-clinics directed by trained psychiatrists have arisen that in fact target preferentially early mood disorders. But in order to conceal this purpose, and above all to avoid being branded as a kind of insane asylum, these institutions have assumed the 'mask of water-cure clinics.'"[55]

Yet the family might see the water-cure clinic as preferential to the asylum. And the director of a well-known private asylum in Bendorf-on-Rhine urged his colleagues to make referrals to such clinics: "[In a water-cure clinic] there are in fact numerous mental patients, and we know from experience that many are discharged as being well." The writer thought separation from the family the main factor in these successes.[56]

The water-clinic Marienberg in Boppard-on-Rhine was typical in every way of the middle-class hydro, and statistics from Marienberg show it was, without question, a primarily psychiatric facility. Of 1,185 patients seen there between 1883 and 1888, only one-fifth were somehow off the psychiatric spectrum (chest disease, anemia, overweight). Fifty-two percent of all patients at Marienberg had "neurosis" (neurasthenia, hysteria, hypochondria); 5 percent suffered from organic diseases of the central nervous system such as neurosyphilis; 13 percent were alcoholics; and 9 percent had some form of what was considered "psychosis," mainly obsessive-compulsive disorders and depression.[57] Thus Marienberg's patients were overwhelmingly psychiatric, and there is no doubt that by subsequent standards Marienberg's

director Karl Hoestermann would have been considered a psychiatrist although he styled himself a hydrotherapist. Places like the Marienberg hydro must be drawn into any comprehensive history of psychiatry, for it was there that the middle classes received their psychiatric care.

By the turn of the century, the water-cure clinics had begun to lose their cachet, because the public, as one observer put it, “thinks of them as an ostensible entryway to the insane asylum.”[58] It was time for middle-class psychiatric care to move on to other kinds of institutions where one would not be so readily hallmarked “insane.”

Tired Nerves and the Rest Cure

At this point, the Americans rush on stage, defining a new disease, neurasthenia, and discovering rest as the means for its cure. “Tired nerves” and the rest cure were both American inventions. But they resounded upon the international scene, legitimating a new kind of open asylum for the care of psychiatric illness and drawing the attention of physicians to psychological means of effecting a cure.

Psychiatrists’ vocabulary for describing the minor psychiatric ailments had really been rather limited. Nerves, hysteria, and hypochondria were all holdovers from the eighteenth century and more or less interchangeable: hysteria for women who had psychosomatic complaints or who seemed emotionally incontinent; hypochondria approximately the same for men; and nerves for everybody who experienced depression, compulsive behavior, or anxiety. These were all considered to be functional nervous diseases, meaning that they were hypothetically organic but demonstrated no tangible tissue changes.

Then in the second half of the nineteenth century, progress in clinical neurology started to make it easier to find something organic while the patient was alive, not just at autopsy. After the 1860s, multiple sclerosis could be distinguished from hysteria, or general paralysis from hypochondria. If the patients toppled over after you asked them to stand up straight with their eyes closed (Romberg’s sign), they probably had neurosyphilis, not nerves.

These new discoveries made a number of physicians very uncertain. As the

old Viennese clinician Salomon Federn (father of psychoanalyst Paul Federn) later put it, “There were all these new means of investigation, such as the reflexes, the different pupil reactions and the complicated examination of skin and muscles which were supposed to point to pathognomonic symptoms [symptoms specific to a given disease]. Many of these new diseases were still little known in the literature, and at the beginning there was probably no neuropathologist, to say nothing of a clinician, who was familiar enough with these processes to be able to make a certain diagnosis.” [59] A weasel word was needed.

This climate of medical uncertainty about what was really organic and what was just hypothetically organic was a mirror image of patients’ uncertainty about what was madness and what was nerves. Both doctors and patients required a bridge between these two uncertainties, an organic-*sounding* disease term to explain psychiatric-*looking* illness behavior. In 1869, New York electrotherapist George Beard supplied this bridge with his announcement of the discovery of neurasthenia, a supposedly distinctive disease entity. Neurasthenia, like its grandchildren chronic fatigue syndrome and multiple chemical sensitivities a hundred years later, served as a bridge between supposedly organic causes and symptoms involving mood and cognition. Beard declared that a large number of nervous symptoms were really owing to a physical exhaustion of the nerves, or neurasthenia. Unlike neurosyphilis, one could not see this physical exhaustion under the microscope, hence it was functional. But it must be real because the patients themselves seemed so genuinely afflicted. Thus neurasthenia became the prototype of the functional nervous diseases.

The range of symptoms the new diagnosis encompassed was vast. Said Beard: “[It] may give rise to dyspepsia, headaches, paralysis, insomnia, anesthesia, neuralgia, rheumatic gout, spermatorrhea [wet dreams] in the male and menstrual irregularities in the female.” What was the cause, Dr. Beard? “... The central nervous system becomes dephosphorized, or perhaps, loses somewhat of its solid constitutents.”[60] Beard’s neurasthenia did not rest on the solidest of scientific foundations.

Yet the new diagnosis became wildly popular, particularly after Beard wrote a lengthy book on it in 1880. (He likened himself to the explorers of Central Africa, pushing into “an unexplored territory into which few men enter....”)[61] The book was immediately translated into German the following year and later appeared in numerous other languages. Until World War I,

Beard's neurasthenia would be the standard diagnosis for all functional nervous diseases, lying as it did between major depression and psychosis on the one hand, and "hysteria," a term still preferred for women, on the other. Neurasthenia became the needed medical weasel word. As Federn said, "Neurasthenia dominated the realm of the chronic functional organ diseases just as the doctrine of bacteriology dominated that of infectious diseases."[62]

But let us say that we have a neurasthenic patient. What are we going to do for him or her? Here a second American nerve doctor weighed in, Silas Weir Mitchell, who created in 1875 a therapy called the rest cure that had almost as historic a trajectory as the disease of neurasthenia, which it was designed to combat.

Although Weir Mitchell's rest cure was probably the most famous rest cure in the history of psychiatry, it was by no means the first treatment prescribing rest. The therapeutic use of rest and isolation for nervous disease had a long previous history. In 1787, William Perfect had placed in his own home a psychotic male patient: "I ... forbade all sorts of intercourse with his relations and acquaintances ..." Perfect confined the patient "to a still, quiet and almost totally darkened room. I never suffered him to be spoken with ... nor permitted by anyone to visit him." Not only did Perfect establish complete authority over this patient, isolating him from outside influences, Perfect also insinuated various dietary and physical therapies into the regimen, such as meals that were "light, cooling and easy of digestion" or warm footbaths. After four months of such treatment, the patient was restored to "a state of sanity."[63] This was Mitchell's rest cure in embryo. In the treatment of melancholia, there was a long Continental tradition of bed rest in a darkened room and seclusion from noise.[64] The notion of isolating asylum patients from friends and family was also very familiar.[65] Historically, these are techniques that each generation of psychiatrists invents for itself.

Mitchell merely described a variant on this age-old theme, but in 1875 it was an idea whose time had come, for the rest cure was ideally conducted in private clinics, and such clinics were just beginning to spring up. Born in 1829, Mitchell graduated in medicine from the University of Pennsylvania, then studied for a year with Claude Bernard in Paris. During the U.S. Civil War, he served as a surgeon with the Union army, at this point acquiring a lively interest in the nervous system as he encountered such phenomena as phantom pain in soldiers whose limbs had been amputated. But Mitchell had an arrogant pretentiousness that ill qualified him for military medicine and

suited perfectly the needs of nervous female patients in the carriage trade. When he returned to Philadelphia, it was as a society nerve doctor that he made his name:

> In January 1874, a Mrs. G from Maine became a patient at a private clinic that Mitchell helped run, the Infirmary for Nervous Diseases. She was suffering from deep exhaustion and was unable to walk up stairs, read, or write. “Any such use of the eyes caused headache and nausea.” After unsuccessful rounds of spa treatment, she had accepted a life of isolation. But there was one positive note: “She was able partially to digest and retain her meals if she lay down in a noiseless and darkened room.”
>
> “I sat beside this woman day after day,” said Mitchell, “hearing her pitiful story.” He found it interesting that she could eat.
>
> “Yes,” she said. “I have been told that on that account I ought to lie in bed.” But she disliked all this bed rest and pleaded with Mitchell not to put her to bed again.
>
> Nonetheless he did so, whereupon, challenging his authority, she began throwing up all her meals. Now it struck Mitchell, not that there was a complex authority issue in the doctor-patient relationship at play here, but that she needed exercise. Rest plus exercise were required, he told himself. “How could I unite the two?” Then it occurred to him: Rubbing! She needed “exercise without exertion.” So he trained a young woman to become a masseuse and rub Mrs. G. A couple of days later, Mitchell hit upon the idea of using electricity as well. “Meanwhile, as she had always done best when secluded, I insisted on entire rest and shut out friends, relatives, books and letters.” In 10 days, Mrs. G “blossomed like a rose.” As Mrs. G was now able to keep her food down, Mitchell began “overfeeding” her with a diet heavy with milk fats. In two months, she gained 40 pounds and went happily home to Maine.[66]

Thus the Weir Mitchell rest cure was born, involving the components of seclusion enforced through bed rest, a milk diet, electrical treatments, and massage. Mitchell believed that his famous cure represented an organic treatment of an organic condition, and throughout his life he was tin-eared to the notion that a psychological component might be involved. He did concede that, for its successful conduct, the rest cure required a “childlike obedience”[67] and therefore worked better with females than males. Yet for Mitchell, organicity was the core: We’ve got to get the blood flowing again

to those “spinal ganglia in the state of exhaustion,” he explained to his colleagues in April 1875, the first outing of the rest cure in the medical community. In 1877, Mitchell published Fat and Blood, explaining the mechanics of the cure. The book became a sensation, and Mitchell’s Infirmary for Nervous Diseases turned into a “Mecca for patients from all over the world.”[68]

The rest cure required a good deal of money and was mainly restricted to an international elite of nervous patients, criss-crossing the oceans in search of relief. Physicians would ship patients far afield to a spa clinic for a rest cure of typically six weeks to three months. One of Mitchell’s first patients was a referral from Hermann Weber. “She was entirely cured,” said a surprised Weber.[69] Because the rest cure was so ideal for a private sanatorium, a symbiotic relationship soon developed among the spa clinics, the urban nerve doctors, and the nervous middle-class females of the Atlantic community. All flourished together: the private clinic boom of the late nineteenth century, the diagnosis of neurasthenia (the rest cure’s target par excellence), and the profession of office-practice psychiatry (not called that).

In 1881, society gynecologist William Playfair, his office on Curzon Street in London’s Mayfair district, gave the rest cure its proper introduction in England: “I have simply followed Dr. Mitchell’s directions, but with results so astonishing and satisfactory to myself, in cases which were quite heart-breaking from their obstinate resistance to all ordinary management, that I am confident [Mitchell’s plan] deserves a more extended trial.”[70] Playfair apparently used private lodgings in London for the conduct of the cure, hiring the Amazon nurses needed to enforce obedience. His colleagues relied on nursing homes and “hysterical homes,” or private clinics, which were better equipped to compel isolation, coordinate electrotherapy and massage, prepare the special diet, and then, toward the end of the cure, to reintroduce the patient to society at the common dinner table.[71]

Charcot brought the rest cure to France in 1885, though he did not acknowledge Mitchell’s priority and insisted that he himself had discovered what he called the “isolation” cure.[72] He would refer patients to a network of water-therapy clinics and the like, then follow them from his seventh-arrondissement consulting room. The highly receptive German doctors required no official introduction, and by 1884 many private clinics had incorporated the “milk diet” *(Mastkur)*, as some called it, or “Mitchell-Playfair Cure,” into their palette of therapies for hysteria and neurasthenia.[73]

By 1900, the rest cure had become the treatment of choice for neurasthenia everywhere, for those who could afford it. The nicest of the new open asylums, nerve clinics, and general sanatoriums now sprouting in many countries would customarily feature the Weir Mitchell treatment. Although neurasthenia as a diagnosis never caught on in the United States as in Europe, it did have a certain vogue in connection with the rest cure. At "The Retreat" in Des Moines, Iowa, a private hospital for "nervous and mental" patients, "neurasthenic and mild mental cases" could obtain a treatment consisting of "rest, baths, massage, electricity ... and suitable trained nurses." The "Crystal Springs Sanitarium for Nervous and Mental Diseases" in Portland, Oregon, claimed to relieve "nervous states, notably the insomnia of neurasthenia," with electrotherapy and similar treatments in "cottage homes."[74]

In Central Europe, the choice was limitless. The Marienberg water-clinic in Boppard made much of its "Playfair Cure" in the treatment of neurasthenia. At Richard Jaenisch's sanatorium in Wölfelsgrund, from among many therapies neurasthenics could choose the code-named "milk diet."[75] The rest cure was thus adopted worldwide.

But it soon became apparent to many doctors that the essence of the Weir Mitchell cure was the physician's authority, and not the specific physical components of the cure itself. What made patients better, in the view of contemporaries, was the act of submission to the doctor. This was a psychological not a physiological component. Harley Street nerve doctor Alfred Taylor Schofield described one of his "neuromimetic" patients whose nerve power had so seeped from her that she was unable to walk. He ordered that she be driven to some secluded gardens and propelled along by two nurses. "That night the patient tried to jump over the banisters and break her leg so that she could not walk; and, failing to do so, refused all food and actually had to be fed for a fortnight by tube through the nose before she would give in. She tore the dress of one nurse to ribands, but at last was overheard one Sunday morning whispering to herself: 'Annie, you've met your match'; and then out she went and soon walked three miles on end."[76] The entire rigmarole was intended to produce in the patient a confession of surrender to medical authority. That it was highly effective reminds us how different the social climate of the day was from our own.

In practice, the rest cure worked as the Anglo-American actress and novelist Elizabeth Robins explained in a fictionalized account of a six-week isolation cure that "Dr. Garth Vincent" (in reality a West End consultant

named Vaughan Harley) put her on.

> "No letters, no telegrams, no messages, no daily papers, no communication of any sort for six weeks," he instructed. Exceptionally, she was attempting the rest cure in her own house, supervised by a nurse.
>
> The nurse arrives and is uneasy about whether she should let "Katharine" read. "It is very seldom [Dr. Vincent] consents to take a real Rest Cure patient that is not nursed at the Home."
>
> Why is that?
>
> "Because—well you see, at the Home everything goes by rule, it's all like clockwork. He thinks that people, especially women, haven't enough sense of discipline to carry out orders for themselves."
>
> Then Dr. Vincent arrives for a visit. "He had come swiftly in, seemed not even to look at her, went straight to the nearest window, already down an inch from the top, and pulled it open a foot and a half. 'You're too hot in here,' he said, and stood an instant taking a stethoscope out of his pocket and fitting it together, frowning down upon it. Katharine was conscious of a little shudder passing over her."
>
> There were struggles over food, for Katharine had been eating little. That evening Katharine had not eaten her butter. The nurse warned that Dr. Vincent might have to return.
>
> "Come here? again? to-night?" asked Katharine.
>
> "Yes," said the nurse. She'd have to phone him.
>
> "Did you ever have to do that?"
>
> "Oh yes," said the nurse.
>
> "What happened?"
>
> "He is always in one of his black rages, when he's sent for."
>
> Katharine began spreading the butter on her toast.
>
> After six weeks of struggles over massage, milk diet, and so forth—all of which Katharine loses—her neurasthenia gets better. She learns that she can get up and look at her letters tomorrow. How excited she is at the thought of going out, her old symptoms now cast behind her. At the end of the story she cries out joyfully "I'm a new creature."[77]

As a physician, one would have to be psychologically unminded indeed to care for such a patient and still believe neurasthenia an organic disease of the nervous system curable with a milk diet. Katharine clearly owed her improvement to her psychological surrender to the medical martinet Dr.

Vincent. As the physicians of the United States, France, Britain, and Germany administered thousands and thousands of such cures, a collective lightbulb started to go on: They were dealing with a disorder having a major psychological component.

Just before going down from Paris to his new practice in Nice, Charcot's student Fernand Levillain, who deemed himself a physiatrist and "clinical neurologist," wrote a precis of the Weir Mitchell cure for French physicians. The specific components such as electrotherapy were really secondary, said Levillain. The central component of the rest cure was isolation from the outside world, for the conferral of "psychological force ... against certain psychological forms of neurasthenia."[78]

In the United States, as well, practitioners began to recognize the psychological component. Neurologist Francis Dercum, professor of nervous diseases at Jefferson Medical College in Philadelphia, was on the whole a big believer in organicity. Yet he thought the rest cure mainly a phenomenon of suggestion. Full-feeding and the other features served mainly to "untie" the patient's "pathological associations," Dercum declared at a Boston psychiatric meeting in 1908.[79] Soon thereafter, another doubting neurologist, Harvard's George Waterman, speculated that Weir Mitchell's own strong personality was probably the curative factor in his famous cure. "The general result ... owes its success rather to the suggestive influence than to any physical change that takes place."[80]

In Britain, Edwin Bramwell, a member of Edinburgh's famous medical dynasty of that name and lecturer on neurology at the university, debunked this supposedly neurological cure designed by America's most famous neurologist: "Even at the present time," Bramwell said in 1923, "there are many who fail to recognise that when isolation is employed in the treatment of the neuroses, it is merely an adjuvant.... In the majority of cases in which a rest cure is advised, the purpose of isolation is to give the physician free play, so that the impression which he makes by the persuasive measures he adopts may not be weakened by countersuggestion." [81] It is important to emphasize that all these thunderers against the organicity of the rest cure were neurologists, not psychiatrists. They were not long in drawing the conclusion: If the rest cure was psychological in nature, then neurasthenia could be treated with psychotherapy.

The significance of the rest cure, neurasthenia, and the rise of private clinics supposedly for medical and neurological disease, is that these

apparently organicist concepts helped open the way for psychotherapy and broke ground for the insight that certain forms of psychiatric illness may yield to the healing power of the human voice. This is why the rest and isolation cures claim such historic significance: They treated the mind in the context of a one-on-one relationship between doctor and patient. The biological model, as it had evolved by the end of the nineteenth century, was much lacking in any understanding of the mind as the intervening link between brain and behavior. Light on the importance of this link started to glimmer in the private clinics as doctors saw such placebos as a milk diet completely transform patients' lives.

Neurology Discovers Psychotherapy

Psychiatry outfitted with biological assumptions bore a strong resemblance in those days to neurology. Not that in office practice it mattered so much. In the absence of qualifying exams for either discipline, physicians were pretty much what they called themselves. In practical terms, a "psychiatrist" or alienist was someone who had spent a good deal of time in asylums, a "neurologist"—the original term meant a specialist in the anatomy of the nerves[82]—someone who had trained in general pathology and internal medicine. Psychiatrists were clearly identifiable as an asylum-based speciality: Of 124 physicians attending the annual meeting of the American Medico-Psychological Association in 1910, for example, only 4 were not identified with an asylum or private nervous clinic.[83] Yet from early on, neurologists too constituted an identifiable speciality, based in private offices rather than institutions. American neurologists formed their own organization in 1875, German neurologists in 1907.[84]

What interested these early neurologists was not so much "nerves" but the neurological implications of such diseases as thyroid deficiency, beriberi, stroke, kidney failure, and the like. Yet they were obliged by their patients to attend upon nerves, nervous disorders being far commoner and more remunerative than the neurological complications of uremia. It was the patients' fearsome rejection of anything "psychiatric" that forced neurologists willy-nilly to become involved with the world of psychoneurosis. If the mad-doctors were unsuitable for the middle classes, the nerve-doctors would have

to serve.

Thus from the very beginning the cure of psychoneurotic illness had a heavily neurological slant. Freud, Janet, and Charcot, the great names in the understanding of hysteria, were all neurologists. The private nervous and organic clinics that after the 1880s took the baton from the water-treatment clinics were virtually all premised on the assumption of treating organic nervous disease and were staffed by physicians whom the public believed to be neurologists. What anxious turn-of-the-century nervous patient would not be comforted to hear, entering the "Sanatorium Friedrichshohe" in Wiesbaden, that Richard Friedländer, owner and chief physician, treated "hypothyroid disease, St. Vitus Dance, peripheral and central paralysis, tabes, neuritis, loss of muscle bulk, and morphinism." Of course Dr. Friedländer also accepted cases of "nervosity, neurasthenia, hysteria, hypochondria, and melancholic depression." We offer, said Friedländer, all of the water therapies, thermal therapy, pine-needle baths and many kinds of other baths, electrotherapy of various kinds, massage and physiotherapy. It all sounded so marvelously organic, especially when he added, "The mentally ill are absolutely excluded from admission."[85]

One of Dr. Friedländer's therapies, however, did not sound so organic. "Psychological influencing plays a cardinal role in the treatment," he said. It was indeed physicians like Friedländer, essentially neurologists, who dipped into psychotherapy rather than mainline psychiatrists. The reason was that neurology offered patients the necessary fig leaf: We believe your problems are organic but we may be able to help you with this new kind of influencing. This cover permitted neurologists to take over the minor psychiatric illnesses under the guise of treating somatic complaints of the central nervous system.

Psychotherapy arrived in medicine by two routes, neither of them particularly psychiatric. Initially, the early hypnotherapists convinced physicians that patients' symptoms could be abolished through suggestion, both hypnotic and nonhypnotic. Then along came the psychological-milieu therapy of the private "neurological" clinics.

It is actually the last chapter of the hypnosis story that concerns us here.[86] Medical hypnosis had begun at the end of the eighteenth century with Franz Anton Mesmer and his French followers. Hypnotism then experienced various ups and downs over the course of the nineteenth century and was finally headed for oblivion, just as the French medical community revived it in the 1880s (it would vanish largely from the medical screen in the decade

before World War I). The revival in France involved two rival groups of physicians: Jean-Martin Charcot, leader of the Salpêtrière school, and Hippolyte Bernheim of the Nancy school, Nancy being the capital of the eastern French province of Lorraine whither the Faculty of Medicine of Strasbourg had fled after the Germans annexed Alsace in 1871. Charcot believed hypnotizability to be evidence of hysteria, seeing no therapeutic use for it save confirming the diagnosis that the patient was indeed hysterical. On the other hand, internist Bernheim believed that hypnosis could be used for medical therapy, there being nothing specifically hysterical about it (Bernheim thought the phenomenon of suggestion to be characteristic of all psychoneuroses: If patients could be suggested into them, they could also be suggested out of them). Bernheim, who had instructed himself in medical hypnotism under the tutelage of an old country doctor named Ambroise-Auguste Liébeault, applied hypnotism with varying degrees of success in a number of organic and psychological conditions. But he soon realized that nonhypnotic suggestion—meaning a good talking-to-was just as effective, particularly among middle-class patients who resisted the pretentious imposition of medical authority that full-blast hypnosis required. After 1883, Bernheim began preaching nonhypnotic suggestion as well, marking the true beginning of modern medical psychotherapy.[87]

The next chapter of the story moves to Amsterdam. A young Dutch medical student named Frederik Willem van Eeden had gone to Paris in November 1885 to collect material for a dissertation on tuberculosis. He happened to audit some of Charcot's lectures and became highly enthusiastic about hypnotic suggestion. After graduating from Amsterdam in 1886, van Eeden returned to Paris, and went also to Nancy. "I had now seen," he later wrote, "that the body could be cured by the mind, and this I felt to be the only true and lasting cure."[88] Back in Amsterdam again, van Eeden met up with another enthusiast of hypnotism, Albert Willem van Renterghem, who had just been to France to visit Liébeault.[89] Van Renterghem had set himself up with a hypnotherapy practice in a small Dutch town and had been run off his feet by avid patients. Later that year, van Eeden and van Renterghem decided to found in Amsterdam an outpatient hypnotherapy clinic, and in August 1887 their "Clinic for Psychotherapeutic Suggestion" opened its doors, van Renterghem handling the business side, van Eeden doing the hypnosis.[90] The clinic thrived although van Eeden, who treated the poor free of charge in any event, refused to collect fees from the rich either. He left it after seven years

to retire from medicine and become an author. This Amsterdam clinic, which evidently employed only hypnosis, represents the first modern use of the term "psychotherapy."[91]

After van Eeden, psychotherapy in the form both of hypnosis and nonhypnotic suggestion began a tour through the world of psychoneurosis.[92] Psychotherapy at this point was not at all incompatible with biological psychiatry for it spoke to the issue of treatment rather than causation. A more dyed-in-the-wool organicist than August Forel, Zurich psychiatry professor between 1879 and 1898, would be hard to imagine. Forel spent much of his time doing neuroanatomy, and his correspondence with colleagues reflects far greater interest in frog brains than in clinical psychiatry. Yet Forel was a master hypnotist.[93] So great was his reputation that one colleague referred to him a woman whom another hypnotist had put into an evil hypnotic trance, with the request that Forel lift the trance.[94] Later in life, Forel even went beyond hypnotism to talk about the therapeutic uses in the doctor-patient relationship of "love" and "intimate knowledge" of patients' lives. Yet at the same time, he referred to the "pathology of brain life."[95] Thus for Forel, there was no contradiction between a neuroscientific view of psychiatry and psychotherapy.

Outside the universities as well, psychotherapy spread among doctors identified with the organic side of psychiatry and with neurology. But it was not the psychotherapy of later "systems," such as family therapy, group therapy, depth therapy, and the like. At its birth, psychotherapy was understood in its Bernheimian sense as the therapeutic use of the doctor-patient relationship, applied in an intimate and informal context.

In Central Europe, with its great concentration of private clinics, psychotherapy first caught on in the open hospitals for nerves and the general sanatoria, almost all of them run by neurologists and internists. Heinrich Obersteiner in Vienna, a psychiatrist so neurologically oriented that he donated to the university a laboratory for brain research, seems to have been among the first to apply hypnotism. He did so at his elegant private nervous clinic in the Viennese suburb of Ober-Dobling, going public on the subject in 1885 with an address to Vienna's "Scientific Club."[96] Six years later, he explained his conception of "psychotherapy" as basically "calming and diverting" the patients, adding that the staff also did some hypnotism and suggestion on nervous patients but that psychotic patients were accessible to

neither.[97] Richard von Krafft-Ebing evidently used “psychical therapy” at the private nervous clinic he had founded in 1886 in a suburb of Graz.[98]

Many smaller clinics followed in the footsteps of these two influential Viennese organicists. Rudolph von Hosslin, who had studied with Charcot and then served as an assistant in the university psychiatric department in Munich, used hypnotism as early as 1887 at the “Neuwittelsbach” clinic he had opened in a Munich suburb.[99] Karl Gerster featured hypnotism at the sanatorium he founded in 1893 in Braunfels-on-Lahn (later he turned to psychoanalysis).[100] The notion of “psychic treatment” was becoming so chic that staff physicians of a private asylum in Berlin could use it as a label for entertaining patients with concerts and amusements. [101] By the mid-1890s, psychotherapy in one or another of its senses had arrived in the private clinics in Central Europe.

In France, with its lack of private clinics and emphasis on state medicine directed from Paris, psychotherapy burst first into such centers as the Salpêtrière. Yet a word of caution: The tradition of moral therapy in French psychiatry ran so deep that the French did not need to import “psychotherapy” from a couple of very junior Dutch hypnotists. Morel, the degenerationist, had invoked in 1857 “the phrase moral therapy that we employ in our asylums to define the action that the physician seeks to exercise on a portion of the degenerates....”[102] Two decades later, physiologist Claude Bernard conducted an investigation of hypnotism, by implication an informal form of psychotherapy. Starting in 1881, Amédée Dumontpallier taught hypnotism at the Pitie Hospital.[103] Nonhypnotic suggestion would enter Paris medicine in 1888 with a “Clinique de Psychothérapie” that Edgar Bérillon, a pupil of Dumontpallier’s and follower of the Nancy school, opened in the rue Saint-André-des-Arts on the Left Bank.[104] Dumontpallier and his students, it must be emphasized, were neurologists and internists, not psychiatrists.

The great French neurologist Charcot had little interest in one-on-one relations with his patients, but after he died in 1893 psychotherapy moved into the medical wards of the Salpêtrière. Pierre Janet, a psychology graduate, had come to the Salpêtrière under Charcot in 1890 to do psychological studies. Janet became a medical student (MD, 1892) and went on to a career as a psychologically minded neurologist. He left the Salpêtrière in 1895 to teach at the College de France although he retained a clinical connection in

the service of neurologist Fulgence Raymond, Charcot's successor. In the Charcot days, Janet was certainly interested in psychotherapeutic approaches to the neuroses, though doubtless out of deference to the master he used the phrase "psychological treatment" rather than psychotherapy. He also relied heavily on hypnotism. [105] So Janet is really the first big name in France in the history of psychotherapy.

The second big name, Jules-Joseph Dejerine, came to the Salpêtrière in 1895, the year that Janet left. Dejerine, 46 at the time, had been born in Geneva but was trained in medicine in Paris and served in various hospitals as an internist and as a neurologist in the true sense, not the German sense, doing research on polio, diseases of the spinal cord, and the like. Yet he was drawn to psychoneurosis in the way of most other neurologists of his day, simply because the patients sought out such doctors, and because much psychoneurosis has what neurologists like to call a pseudoneurological presentation: It looks and feels like organic disease.

What probably drew Dejerine to psychotherapy was the personal interest he took in his patients. One American physician—Smith Ely Jelliffe of New York—recalls Dejerine as "a great big, simple-hearted fellow weighing 250 pounds, 6 feet 2 inches in height," known for his "gusto and a laugh and not afraid of a Rabelaisian reference."[106] Jelliffe found Dejerine's probing of the emotional life of his patients akin to Sigmund Freud and Josef Breuer's "cathartic therapy" (see p. 157).[107] Dejerine did little more than articulate as psychotherapy his own natural clinical style: The sympathetic expression of interest in patients plus the willingness to take time with them and let them talk were the essence of his technique.

After arriving at the Salpêtrière, Dejerine turned Charcot's famous "hysterics' ward" in the Pinel wing into an "isolation service" for a modified version of the rest cure. This involved keeping the curtains drawn about each patient's bed, as well as providing enforced rest, hyperfeeding, and psychotherapy. Dejerine's isolation service marked the first time that the normally very expensive Weir Mitchell rest cure became available outside the setting of the private clinic. It met with great success.[108] Yet the secret to Dejerine's success was not the rest cure as such but his patient and attentive listening to what sick people had to say. He describes his morning rounds on the hysteria ward: "The psychotherapeutic method that I employ has nothing in particular about it. It is as simple as can be, for it is based on reasoning and persuasion, supported by firm but benevolent discipline. On morning rounds I

ask each patient how she spent the night. I explain patiently to her that the symptoms of which she complains do not have the significance that she attributes to them. And I do not go on to the next patient until I see by her answers that conviction is sprout ing in her mind."[109]

Jelliffe had watched Dejerine at work: "He would sit on the edge of the bed with the poor little seamstress or the little cellar rat ... and go over their life's history, their family troubles, difficulties in collecting their bills, how the children's teething kept them up all night, and so on and so on. He poured out a sympathetic emotional type of reaction to them. He was the indulgent humorous father, and the hospital the warm embracing mother with its staff of nurses trained by him."[110] (Jelliffe had become a psychoanalyst by the time he wrote these lines.) It was Dejerine's deliberate use of one of the standard components of the doctor-patient relationship—the benevolent expression of interest—that made his form of psychotherapy well known abroad, especially in England.

When Raymond was chosen instead of Dejerine as Charcot's successor, Dejerine had something of a nervous breakdown.[111] In his misery, he turned for help to a boyhood friend from Geneva who in the meantime had become professor of neurology in Berne: Paul Dubois. Whereas Dejerine had drifted toward Paris, Dubois earned his MD at Berne in 1874, going on in internal medicine. Having acquired a large reputation as an internist, Dubois became involved with electrotherapy, the perfect placebo for an internal-medical practice. So prestigious did he become as an electrotherapist that in 1902 Berne created a chair of neuropathology for him.[112] Although Dubois lived in German-speaking Switzerland, his professional horizons were turned toward France, and he and Dejerine remained close friends, influencing each other over the years (which is my justification for discussing Dubois in the section on France). Dejerine was said to have taken his psychotherapeutic ideas from Dubois. In 1904, two years after gaining his professorship, Dubois published (in French), what was to become the most influential book on psychotherapy written before Freud. Dubois offered a highly rationalistic philosophy of "persuasion," using the doctor-patient relationship to persuade the patient to change his or her ways in a kind of Socratic dialogue in which medical advice constantly tugged patients toward betterment. Dubois had no use either for hypnotism or for the notion of an unconscious mind. "For neurasthenia," he said, "there is another psychotherapy altogether [than Bernheim's suggestion], a psychological training that does not try to conjure

away fatigue but to make it disappear by slowly suppressing its principal cause: emotivity."[113] The "rational psychotherapy" of Dubois was a bit too preachy even for Dejerine's liking.[114] But the names of Dubois and Dejerine are linked together as offering the principal form of psychotherapy available before Freud.

This horde of neurologists and internists rushing into the private practice of psychotherapy touched off a terrific turf struggle in France. In 1911, Dejerine, just having received the Charcot chair of neurology, tried to reach out to the younger generation of family doctors, assuring them that through such tactics as putting the patient at ease and expressing sympathy, they would be able to get the "neuropaths" back on their feet.[115] Gilbert Ballet, the professor of psychiatry, would have none of these neurological pretentions: The psychoneuroses belong to psychiatry, he claimed. "Thus, gentlemen," said Ballet, "the field of psychiatry embraces the study of all the disorders of the mind *[psychisme intellectuel]"* and everything related to these disorders. Psychiatry, claimed Ballet in prophetic words, must not be limited to the study of psychotic illness. Whatever brain mechanism neurotic illness possessed was unimportant. As long as the symptoms were "psychic," the patient belonged to psychiatry.[116] To this cool challenge, Dejerine responded rather lamely that psychiatry should limit itself to mental illness, while neurology would take the "neuropaths."[117] The exchange is exquisite in its foreshadowing of the future: Ballet's views would ultimately win, conquering for psychiatry the garden-variety neuroses of modern life. Dejerine's views would ultimately lose, as neurology became a rarified specialty dealing, it is only a slight exaggeration to say, with unusual and incurable diseases of the central nervous system.

Yet in the years before World War I, it was the neurologists who triumphed by acquiring office-practice psychotherapy. Private-practice psychotherapy, the basis of late-twentieth-century psychiatric practice, began with the neurologists, not the psychiatrists.

Harley Street in London and Queen Street in Edinburgh became Britain's psychotherapeutic epicenters. Although the British shunned specialization well into the twentieth century, Dejerine and Dubois were widely read and cited among British physicians who styled themselves simply as consultants with no mention of specialty. Just as soon as Edinburgh internist Byrom Bramwell had read of Dejerine's therapy in 1903, he astonished the medical students by putting the new psychotherapy into practice to cure a patient with

a hysterical paralysis. After isolation and milk feeding, she was soon hopping about the corridor.[118] By contrast, psychiatry in Britain remained in the asylums virtually until World War II.[119] "The only psychiatrists I can recall in private practice," said psychoanalyst Ernest Jones, "were those who had retired from the position of superintendent at Bethlehem."[120] (Among them, Jones mentioned George Savage, known for having attended Virginia Woolf in bouts of depression.[121]) The distinctive British contribution to office-practice psychotherapy had always been the suggestive effect of a brass plate on a Harley Street door, and would remain so for many years.

Efforts of American neurologists to snare the lucrative carriage trade went back at least to 1879 when William Hammond, a New York neurologist and former surgeon-general, suggested treating "mental aberrations" at home.[122] The big American cities contained many "neurologists" who in fact were the equivalent of today's psychiatrists. It was at the instigation of neurologist-internists such as Lewellys Barker of Johns Hopkins—and not psychiatrists such as Adolf Meyer—that Dejerine and Dubois were enthusiastically received. Barker had visited Dejerine in Paris in 1904, then read up on Dubois and Janet. "During my first year in the [Hopkins] clinic," said Barker, "we had more than eighty cases in which psychotherapy was the main influence in treatment." Barker set out to bring the message to family physicians that nervous patients could be helped.[123]

In the decade before World War I, the new doctrine of psychotherapy spread rapidly among internists and family physicians in the United States.[124] In 1913, Charles Dana, professor of neurology at Cornell Medical College in New York City, looked to the future: Neurology has passed, he said, from the microscope and the autopsy suite to "the study of psychoneuroses." The neurologist now had to contend with "subjective states and the importance to all neuroses of environment, education ... the character, temperament and social conditions of his patients." Why? Because "nervous diseases are so largely social." Therefore, the neurologist should follow his patients from youth, "advise them as to marrying, even marry them at times and tell them about the management of the children." An overwhelming role indeed for specialists who only recently were cutting sections of spinal cords. "He must be a kind of superman," Dana concluded, "one with higher ideals, more potent inhibitions and wiser in life and wider in outlook than those whom he is trying to guide."[125] What lends this grandiosity its humor is that, after

World War II, it was precisely the psychiatrists who began demanding this role.

The initial diffusion of psychotherapy had nothing to do with the discipline of psychiatry. The doctrine of “madness” had driven the patients from psychiatry, the panache of “nerves” luring them to the neurologist and the internist. By World War I, in every country in Western society psychiatry had become marginal to the mainstream of medicine and to the ebb and flow of dysphoria in daily life. Intellectually, it was being gobbled up by medical specialties whose very premise was organicity. To survive as a discipline, psychiatry had to break free of insanity and of organicist assumptions about the nature of “nervous disease.”